TEACHER RESOURCE GUIDE · YEAR 9–10

FOR TEACHER USE ONLY

ADVANCED WATER SAFETY Teacher Resource Guide

Year 9–10 HPE · Victorian Curriculum F–10 Version 2.0 · Companion to the Year 9–10 Student Workbook

Year 9–10 | 5 Lesson Plans | Complete Answer Key | A–E Assessment Rubric | Differentiation | VC2HP10P02 · VC2HP10P08 · VC2HP10P09 · VC2HP10M01

📋 Contents

1. Curriculum Alignment & Achievement Standards
2. Unit Overview & Learning Intentions
3. Lesson Plans (Lessons 1–5)
4. Student Workbook — Answer Key
5. Assessment Rubric (A–E, 6 Criteria)
6. Assessment Checklist (Summative Task)
7. Differentiation & Inclusion
8. Teacher Background Notes
9. Resources & Sources

1 Curriculum Alignment & Achievement Standards

Code	Content Description	Strand	How this unit addresses it

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

VC2HP10P02	Refine, evaluate and adapt strategies for managing changes and transitions	Personal, Social & Community Health	Students investigate how peer influence, adolescent risk-taking behaviour, and social transitions affect exposure to drowning risk; explore social determinants using a structural lens
VC2HP10P08	Plan, rehearse and evaluate strategies (including first aid and CPR) for managing situations where their own or others' health, safety or wellbeing may be negatively impacted at home, school and in the community	Personal, Social & Community Health	Students plan and rehearse DRS ABCD emergency response; evaluate CPR and first aid options; analyse drowning prevention strategies using evidence and data
VC2HP10P08	Plan, rehearse and evaluate strategies (including first aid and CPR) for managing situations where their own or others' health, safety or wellbeing may be negatively impacted at home, school and in the community	Personal, Social & Community Health	Students design community interventions; audit school provision; evaluate accessibility of water safety programs for at-risk groups; propose targeted actions
VC2HP10P09	Critique health information, services and media messaging about relationships, lifestyle choices, health decisions and behaviours to evaluate their influence on their own, others' and community's attitudes and actions	Personal, Social & Community Health	Students evaluate credibility of prevention evidence from LSV, WHO, RLSSA and international sources; compare and apply findings to Victorian decisions
VC2HP10M01	Apply specialised movement concepts in aquatic contexts; analyse performance against criteria	Movement & Physical Activity	Students apply survival swimming theory and rescue technique analysis (pool sessions if available)

📋 Achievement Standard Connections — Levels 9 & 10

By the end of Level 10, students are expected to:

- Critically analyse health information and the reliability of evidence in health contexts
- Plan and evaluate strategies that build individual and community capacity for health promotion
- Develop and evaluate complex health literacy skills including source evaluation
- Apply and refine specialised movement skills; analyse performance using data
- Propose evidence-based community health solutions with measurable outcomes

2 Unit Overview & Learning Intentions

Element	Detail
Unit Title	Advanced Water Safety: Data, Physiology & Prevention Policy
Year Level	Year 9–10 (Ages 14–16)
Duration	5 × 50-minute lessons (expandable with pool sessions via Pool Practical Companion)
Prerequisite	Year 7–8 Water Safety unit (or equivalent foundational knowledge of flags, DRS ABCD, SwimSafe rules)
Assessment	Workbook (Sections 1–5) + Summative Task (Section 6: policy brief, multimodal campaign, or school audit)
Key Departure from Y7–8	Emphasis shifts from knowledge and application to analysis, evaluation, and synthesis. Students are expected to use data, argue positions, and produce evidence-based products rather than identify and describe.

🎯 Know / Understand / Do

KNOW

- Victorian drowning rates vs national/global
- Physiology of drowning and CPR
- Social determinants of drowning risk
- Prevention strategy evidence hierarchy

UNDERSTAND

- Why data alone doesn't prevent drowning
- How social factors amplify or reduce risk
- Why some prevention strategies work better than others

DO

- Calculate and interpret drowning rate data
- Evaluate prevention evidence critically
- Compare international approaches
- Design an evidence-based community product

- International approaches (RLSSA, RLSS, Red Cross)
- The relationship between policy, community, and individual action
- Argue a health policy position

🔍 What this unit does not do, and why

Most teaching resources list what they cover. This lists what they do not, because a limit you know about is something you can plan around and a limit you discover in week three is a problem. Three of these are deliberate scope decisions; one is genuinely undone; one is not something any document can supply.

What is missing	What that means for you	Why it is like that
Most of the HPE curriculum	This unit addresses 10 of the 40 Victorian content descriptions. It services *Safety* substantially and contributes to three other focus areas. It does not touch the other eight.	**Deliberate.** It is a water safety unit, not a program. The Curriculum Coverage Map lists every code it does not address so your planning can place them elsewhere.
Evidence of student achievement	The curriculum codes record what was *taught*. They are not, by themselves, evidence that students achieved anything.	**Deliberate.** Victorian Curriculum 2.0 reports against achievement standards. Evidence comes from the rubrics, work samples and your moderation — not from a coverage map.
Tiered intervention	The scaffolded and simplified versions are Tier 1 differentiation. They are not a Tier 2 or 3 intervention program and should not be counted as one.	**Deliberate.** Intervention is designed around a student, by your school, on data this unit does not hold.
Flexible grouping on evidence	The lessons use whole-class and fixed-pair structures. They do not tell you to form and re-form groups during a lesson based on what you are seeing.	**Genuinely open, and the one gap left.** VTLM 2.0 asks for groups formed on evidence of learning. That decision depends on what is in front of you, so the materials prompt it (see the checks-for-understanding repertoire) but cannot script it.

		Worth adding to your own lesson notes.
Everything that happens in the room	Classroom rules and routines, active supervision, how you respond to a student in the moment, the relationships you build with students and families.	**Not a document's job.** A resource can prompt these; it cannot evidence them. An alignment document that claimed otherwise would be worth less than this one.

Why this section exists at all. This unit was audited in October 2026 against all 14 teaching strategies of the Victorian Teaching and Learning Model 2.0. Twenty technique-level gaps were found; nineteen were closed, and the one that was not is in the table above. The full audit, including what was changed and what the model asks that a resource suite structurally cannot deliver, is in the VTLM 2.0 Alignment Guide. If you are documenting VTLM implementation for an Annual Implementation Plan, read that rather than this summary.

3 Lesson Plans — 6 × 50 Minutes

1 Advanced Data Analysis — Victorian & Global Drowning Trends

⏱ 50 min | Section 1 | VC2HP10P02 · VC2HP10P08 | Classroom

🎯 FOCUS THE LEARNING — 5 MIN

LEARNING OBJECTIVE

Analyse Victorian and national drowning data, and evaluate what over-representation in a population does and does not tell you.

SUCCESS CRITERIA — SHARE THESE WITH STUDENTS

- ☐ I can calculate a rate per 100,000 and explain why it is not the same as a count.
- ☐ I can explain what over-representation means, and state one limitation of the Victorian multicultural figure.
- ☐ I can present data in a graph that is labelled well enough for someone else to interpret without me.

- Open with: "Victoria had 52 drowning deaths in 2024–25. That's 0.75 per 100,000. Australia's national rate was 1.13 per 100,000 in 2025/26. Should we be proud of this, or worried by it? Discuss." Pair discussion (2 min) → class share.
- Ask: "What social factors might explain why Victoria is BELOW the national rate on some metrics yet has particular problems with CALD communities and inland waterways?"
- Distribute workbooks — introduce the expectation shift: "At Year 9–10, describe is not enough. We analyse, evaluate, and justify."

📣 EXPLICIT TEACHING – I DO — 13 MIN

- Teacher-led discussion: "People from multicultural backgrounds account for 37% of Victorian drowning deaths since 2015–16, and country of birth is unknown in another 22% of cases. What does over-representation mean here, how confident can we be in the figure, and how do we explain it without stereotyping?"
- Introduce the concept of social determinants: access to swimming lessons, cultural norms around water, fishing risk, language barriers to safety messaging.
- Reference the LSV multicultural safety page (lsv.com.au/community/multicultural-safety/) as evidence that LSV is specifically trying to address this.

🤝 GUIDED PRACTICE – WE DO — 15 MIN

- Students complete Workbook Activity 1.1 calculations independently: percentage calculation (Q1.1a), rate comparison (Q1.1b), and CALD over-representation analysis (Q1.1c).

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

- Circulate and prompt: “Show your working. What does the number MEAN, not just what it is?” Students have the over-representation concept from the explanation — hold them to using it, not restating the percentage.
- If devices available: students access LSV Drowning Report to verify their figures and source their graph data for Q1.1d.

✏️ INDEPENDENT APPLICATION – YOU DO 12 MIN

- Students complete data visualisation task (Q1.1d): bar graph of deaths by location type. Emphasise: labels, title, axes, interpretation — not just drawing bars.
- Pair check: swap workbooks and verify each other's calculations for Questions 1.1a–c.

📝 MONITOR PROGRESS 5 MIN

- Exit ticket: "In one sentence: what is the SINGLE most important data insight from today's lesson, and why?"

Review the success criteria. Return to the three criteria. The second is the one students most often answer with the statistic rather than the limitation — cold call two students on the 22% unknown figure before you accept it.

📌 Teacher Note — CALD Discussion

Handle the CALD data discussion carefully. The goal is systemic analysis — why do structural factors (limited swim access, language barriers, fishing culture risk) create disproportionate outcomes — not a commentary on any particular community. Frame as: "This data tells us the system hasn't served everyone equally, not that some groups are less careful."

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

2 Physiology of Drowning & CPR Mechanics

⏱ 50 min | Section 2 | VC2HP10P08 · VC2HP10M01 | Classroom

🔁 REVIEW — RETRIEVAL PRACTICE — 3 MIN

- **Retrieval starter — Lesson 1.** No notes, individually, then self-mark (2 min). **1.** What was Victoria's drowning rate per 100,000 in 2024–25? **2.** Why is a rate more useful than a count when comparing two populations? **3.** What share of Victorian drowning deaths since 2015–16 involved people from multicultural backgrounds — and what is the catch with that figure?

Why this is here. Retrieving prior learning from memory, rather than re-reading it, is what moves it into long-term memory. Low-stakes and ungraded — the act of recall is the point, not the score. VTLM 2.0: *Retention and recall*, *Revisit and review*.

🎯 FOCUS THE LEARNING — 5 MIN

LEARNING OBJECTIVE

Explain the physiological sequence of drowning and why CPR works, using the correct terminology.

SUCCESS CRITERIA — SHARE THESE WITH STUDENTS

- ☐ I can describe, in order, what happens physiologically from submersion to cardiac arrest.
- ☐ I can use hypoxia, laryngospasm and cardiac output correctly in a sentence of my own.
- ☐ I can explain why ~25–30% of normal cardiac output is enough to matter, and what the 4-minute window refers to.

- Open with: "CPR generates only 25–30% of normal cardiac output. If normal output keeps you conscious and alive, why is 25–30% enough to bridge survival until an ambulance arrives?"
- Think-pair-share. Draw on whiteboard: normal cardiac cycle vs CPR cycle. Establish prior knowledge baseline.

📣 EXPLICIT TEACHING — I DO — 8 MIN

- Teach the physiological sequence *before* students use the terminology. Build it on the board one step at a time, naming each term as you add it: submersion → breath-hold → **laryngospasm** (involuntary closure of the vocal cords on water contact) → hypoxaemia → loss of consciousness → **cerebral hypoxia** → cardiac arrest.

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

- Pre-teach the six terms students will need: **hypoxia**, **laryngospasm**, **cardiac arrest**, **cerebral hypoxia**, **cardiac output**, **oxygenation**. One sentence each, with an example — not a definition to copy down. Full wording is in Section 9, Physiology Quick Reference.
- Name the misconception explicitly: laryngospasm means water never reaches the lungs in roughly 10–15% of drowning deaths, so *no water in the lungs does not mean the person did not drown*. Students who miss this read the case studies wrongly later.
- Check for understanding before moving on: thumbs up / down on three statements — “Laryngospasm lets water into the lungs” (down), “Cerebral hypoxia is the critical mechanism in drowning death” (up), “Cardiac arrest comes before loss of consciousness” (down).

Why this phase was added. Students previously met six technical terms for the first time inside an independent workbook activity. That is a cognitive-load failure, not a difficulty problem: the vocabulary has to be explained before it is used. VTLM 2.0: *Explicit explanation and modelling*, Practice 1.

🤝 GUIDED PRACTICE — WE DO — 12 MIN

- Students complete Workbook Activity 2.1a (physiological sequence of drowning) and 2.1b (CPR physiology and 4-minute window).
- Key vocabulary to reinforce: hypoxia, laryngospasm, cardiac arrest, cerebral hypoxia, cardiac output, oxygenation.
- Circulate: ensure students are using the vocabulary correctly, not just repeating it.

📣 EXPLICIT TEACHING — I DO (PART 2) — 8 MIN

- Explain secondary drowning — a real but rare risk. Symptoms appear hours after a near-drowning event: coughing, laboured breathing, fatigue, chest pain. Always seek medical review after any submersion incident, even if the person appears fine.
- Walk through the 1-10-1 cold water survival rule with reference to Lake Eildon context. Ask: "Why does the same water temperature affect a 14-year-old vs a 45-year-old differently?" (Body composition, fitness level, clothing.)

✏️ INDEPENDENT APPLICATION — YOU DO — 9 MIN

- Students complete Q2.1c (secondary drowning research) — requires device access.
- Class discussion: "What is the implication for parents, teachers, and coaches who supervise water activities? What should happen AFTER a near-miss?"

📝 MONITOR PROGRESS — 5 MIN

- Exit ticket: "Explain the 4-minute window to someone who has never heard of it — in plain language, not medical jargon."

Review the success criteria. Check criterion two by asking for a sentence, not a definition. Students who can recite “laryngospasm is involuntary closure of the vocal cords” but cannot use it in a sentence have the words without the concept.

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

3 Social Determinants of Drowning Risk

⏱ 50 min | Section 3 | VC2HP10P02 · VC2HP10P08 | Classroom

🔁 REVIEW — RETRIEVAL PRACTICE — 3 MIN

- **Retrieval starter — Lessons 1–2.** No notes, 2 min. **1.** Define hypoxia. **2.** What is laryngospasm, and roughly what share of drowning deaths does it account for? **3.** From Lesson 1: state one limitation of the multicultural drowning figure.

Why this is here. Retrieving prior learning from memory, rather than re-reading it, is what moves it into long-term memory. Low-stakes and ungraded — the act of recall is the point, not the score. VTLM 2.0: *Retention and recall*, *Revisit and review*.

🎯 FOCUS THE LEARNING — 6 MIN

LEARNING OBJECTIVE

Evaluate how social determinants shape drowning risk, and distinguish structural explanation from stereotyping.

SUCCESS CRITERIA — SHARE THESE WITH STUDENTS

- ☐ I can name four social determinants and explain the mechanism by which each raises risk.
- ☐ I can explain the difference between correlation and causation using a water safety example.
- ☐ I can propose a school action that addresses a determinant, and say why it would work.

- Open with the "Values Clarification" exercise: post four corners with signs — "Strongly Agree," "Agree," "Disagree," "Strongly Disagree." Read statements: "Males drown more because they're less careful than females." "CALD communities drown more because of language barriers."
- Students move to their corner and defend. Teacher challenges each position. Point: these are over-simplifications of a complex social issue. Introduce the concept of structural vs individual explanations.

📣 EXPLICIT TEACHING — I DO — 7 MIN

- Teach the framework *before* the table, not after it. Introduce the WHO Social Determinants of Health framework: health outcomes are shaped by the conditions in which people are born, grow, live, work and age.
- Name the four determinants students will meet in the table and give one water safety mechanism for each: **access** (cost and availability of swimming lessons), **language**

(safety messaging that does not reach you), **occupation and recreation** (rock fishing, inland work and travel), **social norms** (what a group treats as normal risk).

- Teach the distinction the whole lesson turns on, explicitly and with an example: a structural explanation names a condition that can be changed ("lessons cost money and are not offered in community languages"); a stereotype names a group and stops there ("they are not water people"). Students will produce the second unless you teach the first.
- Teach correlation and causation here too, because the table will produce both and the success criteria ask for the difference. Worked example: people from multicultural backgrounds are over-represented in Victorian drowning deaths — that is a *correlation*. The causal claim is about mechanism: cost and language barriers reduce access to swimming lessons, which reduces competence, which raises risk. Background is not the cause; the barriers are. Ask students for the mechanism every time they offer a determinant.
- Check for understanding before releasing them to the table: give one statement of each kind and have the class signal which is which.

Why this phase moved. The WHO framework and the correlation/causation distinction previously sat in the Explain phase *after* students had filled in the determinants table. The framework is what makes the table analysable, so it now precedes it. The jigsaw itself is unchanged — it follows as guided practice.

🤝 GUIDED PRACTICE – WE DO — 12 MIN

- Students complete Workbook Activity 3.1a (social determinants table). Pairs — assign different determinants to each pair for jigsaw reporting later.
- Prompt: "Which of the four determinants is this? What is the mechanism?" The correlation/causation distinction was taught in the previous phase — require them to apply it rather than arrive at it.

🤝 GUIDED PRACTICE – WE DO (JIGSAW SHARE) — 8 MIN

- Jigsaw sharing: each pair reports on their assigned determinant (5 min). Teacher synthesises.
- Do not re-introduce the WHO framework here — it was taught in the explicit teaching phase. Instead, synthesise against it: as each pair reports, ask which of the four determinants their finding belongs to and name the mechanism.
- Connect to Victorian data: CALD over-representation, male risk-taking, rural/regional inland waterway risk.

✏️ INDEPENDENT APPLICATION – YOU DO — 9 MIN

- Students complete Q3.1b (3 school actions to reduce CALD disproportionality). Encourage specificity: "Not just 'translate resources' — WHERE would you distribute

them? WHEN? WHO would deliver them?"

📝 **MONITOR PROGRESS** **5 MIN**

- Exit: "Name ONE social determinant that a school can address and ONE that it cannot. Why is that distinction important?"

Review the success criteria. The distinction between structural explanation and stereotyping is the whole lesson. Ask one student to restate the difference in their own words before anyone leaves.

4 Prevention Strategies & International Comparison

⏱ 50 min | Sections 4–5 | VC2HP10P08 · VC2HP10P09 | Classroom + Devices

🔁 REVIEW – RETRIEVAL PRACTICE — 3 MIN

- **Retrieval starter — Lessons 1–3.** No notes, 2 min. **1.** Name three social determinants of drowning risk. **2.** What is the difference between correlation and causation? **3.** From Lesson 2: what does the 4-minute window refer to?

Why this is here. Retrieving prior learning from memory, rather than re-reading it, is what moves it into long-term memory. Low-stakes and ungraded — the act of recall is the point, not the score. VTLM 2.0: *Retention and recall*, *Revisit and review*.

🎯 FOCUS THE LEARNING — 5 MIN

LEARNING OBJECTIVE

Evaluate prevention strategies against evidence, and justify how limited resources should be allocated.

SUCCESS CRITERIA — SHARE THESE WITH STUDENTS

- ☐ I can rate a prevention strategy on evidence strength, target population, limitations and cost.
- ☐ I can identify one element of an international approach that Victoria has not adopted.
- ☐ I can justify a funding decision by citing evidence, not preference.

- Open with: "The government has $5 million to spend on drowning prevention. You have 60 seconds to decide how to spend it. Go." Quick-fire round — write answer on a piece of paper, then reveal and discuss.
- This primes Activity 4.1 (prevention strategy evaluation table and Q4.1a justification).

📣 EXPLICIT TEACHING – I DO — 7 MIN

- Teach the evaluation framework before students apply it. The workbook uses four criteria — **evidence strength**, **target population**, **limitations**, **cost** — and students will otherwise rate strategies on how sensible each one sounds.
- Model it once, out loud, on a strategy that is *not* in the activity so you are not giving away an answer. Suggested: public rock-fishing signage. “Evidence strength — limited; signage changes knowledge more reliably than it changes behaviour. Target population — narrow, and many at-risk anglers do not read English signage. Limitations — it addresses what to do, not why people do not. Cost — low. So: cheap, weak, and worth keeping only alongside something stronger.”

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

- Make the evidence hierarchy explicit: engineering and legislation (pool fencing, mandatory life jackets) generally outperform education, which outperforms awareness campaigns — because the first group works whether or not a person is paying attention. This is the distinction the A-standard in the rubric asks for.
- Check for understanding: “Which is likely to have stronger evidence — four-sided pool fencing or a social media campaign? Why?” Cold call, do not take hands.

Why this phase was added. Activity 4.1 asks for “research and judgement, not just description” against a four-criterion framework students had never been taught. The criteria are the workbook’s own; this phase teaches them before they are used.

🤝 GUIDED PRACTICE – WE DO — 14 MIN

- Students complete Workbook Activity 4.1 (prevention strategy evaluation), applying the four criteria modelled in the previous phase — evidence strength, target population, limitations, cost. Requires research and judgement, not just description.
- Students research for Activity 5.1 (international comparison table) using three organisation websites: royallifesaving.com.au, rlss.org.uk/education, redcross.org/take-a-class/water-safety.

🤝 GUIDED PRACTICE – WE DO (SHARE & SYNTHESISE) — 8 MIN

- Class share: What did students find about international approaches? Key comparison points: UK uses "Float to Live" campaign; Red Cross uses "Think So You Don't Sink" decision framework; RLSSA uses the "Water Safety Framework."
- Teacher highlights: what Australia does well (lifeguard system at patrolled beaches) vs areas for improvement (pool fencing compliance, CALD engagement).

✏️ INDEPENDENT APPLICATION – YOU DO — 9 MIN

- Students complete Q5.1a (identify one unadopted international element — argue whether Victoria should adopt it).
- Prompt: "Evidence-based argument means citing the organisation, not just saying 'I think this is good.'"

📝 MONITOR PROGRESS — 4 MIN

- Think-pair-share: "Which ONE prevention strategy has the strongest evidence, and which has the weakest? Defend your answer."

Review the success criteria. Criterion three is the Year 9–10 standard. “I think this one is good” does not meet it; name the organisation and the evidence.

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

5 Diving, Spinal Injuries & High-Risk Teenage Behaviour

⏱ 50 min | Workbook Section 6 | VC2HP10P02 · VC2HP10P08 | Individual + Discussion

🔁 REVIEW – RETRIEVAL PRACTICE — 3 MIN

- **Retrieval starter — Lessons 1–4.** No notes, 2 min. **1.** Name the four criteria used to evaluate a prevention strategy. **2.** From Lesson 3: name one determinant a school can act on and one it cannot. **3.** From Lesson 1: Victoria's rate and the national rate — which is higher?

Why this is here. Retrieving prior learning from memory, rather than re-reading it, is what moves it into long-term memory. Low-stakes and ungraded — the act of recall is the point, not the score. VTLM 2.0: *Retention and recall*, *Revisit and review*.

🎯 FOCUS THE LEARNING — 6 MIN

LEARNING OBJECTIVE

Explain why diving injuries concentrate in young males, using a structural rather than an individual account.

SUCCESS CRITERIA — SHARE THESE WITH STUDENTS

- ☐ I can explain why the cervical spine is the vulnerable region in a diving injury.
- ☐ I can explain the 95% male statistic without saying that males are simply more careless.
- ☐ I can identify what the three case studies have in common and why that factor is dangerous.

- Display the six-stat data grid (10%, 20%, 95%, 75%+, 66%, +34%) with no context. Ask: *"What do you think these numbers describe?"* Take 4–5 responses before revealing the topic.
- This cold-open approach consistently produces high engagement — students are surprised the topic is not drowning.
- Key question to pose: *"If 95% of patients are male — is that a biology fact or a social fact? What's the difference?"*

📣 EXPLICIT TEACHING – I DO — 9 MIN

- Direct instruction (5 min): Walk through the social determinants explanation for the 95% male statistic. Use the masculinity norms framework from Section 3. The argument students need to be able to make: *this is not biology, it is a social construction that can be changed.*

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

- Introduce the concept of false confidence as a structural problem, not an individual attitude. Reference the Bawley Beach sandbank formation — the person did everything they had learned was safe. The information environment failed them.

🤝 GUIDED PRACTICE – WE DO — 15 MIN

- Students read the three Australian case studies (Terrigal Wharf NSW 2025, Torquay Front Beach Vic 2025, Bawley Beach NSW 2024) individually — 5 minutes.
- Pair discussion (4 min): *"What single factor appears in all three cases? Why is that factor particularly dangerous?"* Students now apply the explanation just taught: ask them to name where false confidence and depth variation appear in each case, not to rediscover the idea. A response of “they dived into shallow water” means the structural account has not transferred — point them back to the board.
- Class discussion (4 min): Draw out the cervical spine mechanism. If you have a foam spine model or skeleton, this is the ideal time to use it. The key physiological point: C4–C6 compression → instant permanent quadriplegia.
- Introduce inland waterway risk factors using the five-factor framework. Students add notes to their workbook.

✏️ INDEPENDENT APPLICATION – YOU DO — 12 MIN

- Students complete Activities 6.1 and 6.2 (case study analysis and myth vs evidence) in the workbook. Circulate and prompt deeper reasoning — push students away from surface-level "they should have been more careful" responses.
- Activity 6.3 (prevention strategy design) can begin in class or be set as homework preparation for Lesson 6.
- For extension students: prompt them to locate one primary source from AIHW or RLSSA before next lesson.

📝 MONITOR PROGRESS — 5 MIN

- Exit ticket: *"What is one specific way the water depth at Bawley Beach could have changed between the last safe visit and the injury — and what should have been done differently?"*
- Correct answer involves sandbank formation and the need to check depth physically every time, regardless of prior experience.

Review the success criteria. Criterion two is the one to check hardest. If students land on “boys take more risks”, the structural explanation has not transferred — re-teach it rather than accepting the answer.

⚠ Sensitivity Note

> If you have students who have experienced aquatic injury (their own or a family member's), be aware this content can be confronting. The case studies are factual but involve serious injury. Brief students at the start that the content involves real injuries, and have a plan for any student who needs to step out.

6

🔁 REVIEW — RETRIEVAL PRACTICE 8 MIN

- Rapid revision Kahoot covering Lessons 1–5 content (use Question Bank resource for Y9–10 equivalent questions). Focus on: rate calculations, physiological terms, social determinants, prevention strategy hierarchy.

Why this is here. Retrieving prior learning from memory, rather than re-reading it, is what moves it into long-term memory. Low-stakes and ungraded — the act of recall is the point, not the score. VTLM 2.0: *Retention and recall*, *Revisit and review*.

🎯 FOCUS THE LEARNING 4 MIN

LEARNING OBJECTIVE

Communicate an evidence-based water safety argument to a specific audience, and justify the choices made.

SUCCESS CRITERIA — SHARE THESE WITH STUDENTS

- ☐ I can select evidence that supports my argument and cite it with a source and URL.
- ☐ I can shape my message for one named audience rather than for a general reader.
- ☐ I can check my own work against the assessment criteria before I hand it in.

- Brief the assessment task and the criteria it will be marked against before students begin drafting. Point them at the Assessment Checklist in Section 6 and the rubric in Section 5.

✏️ INDEPENDENT APPLICATION — YOU DO 28 MIN

- **Option A — Policy Brief:** 600–800 words to Victorian Minister for Sport. Must cite ≥3 sources with URLs, use Victorian data, address a social determinant, and propose measurable outcomes. Scaffold: Situation → Evidence → Recommendations (×2) → Success Metrics.
- **Option B — Multimodal Campaign:** Target ONE high-risk group from Victorian data. Must include: campaign name, key message (evidence-based), visual concept description, target platform, and 100-word justification of design decisions.
- **Option C — School Audit:** Structured report on the school's current water safety provision. Must benchmark against Curriculum 2.0 requirements (VC2HP8P10,

VC2HP8M01) and LSV classroom resources toolkit. Include a priority matrix (urgency × impact).

📝 MONITOR PROGRESS 10 MIN

- Peer review using the Assessment Checklist (Section 6 of this guide). Partners check: evidence present? Audience clear? Call to action specific? Sources cited with URLs?
- Collect completed workbooks. Return with teacher feedback before report writing is finalised.

Review the success criteria. Students rate themselves against the three criteria before submitting, then name the one their draft evidences most weakly. That self-judgement is the self-regulation step.

4 Student Workbook — Answer Key

📌 Answer Key Guidance

Year 9–10 questions are largely analytical and evaluative — there is no single correct answer for most questions. Use the sample answers as **quality benchmarks** for the A-level standard. Accept well-reasoned alternatives with supporting evidence.

Section 6 — Diving, Spinal Injuries & High-Risk Behaviour

Activity 6.1a (common factors): Strong answers identify *false confidence from prior experience* and *unknown/changing water depth* as the common mechanisms — not just "they dived." The mechanism matters: depth changed due to sandbank formation (Bawley), tidal variation (Torquay), and structural shallow zone beneath a wharf (Terrigal).

Activity 6.1b (95% male — structural explanation): A-standard responses use the social determinants framework from Section 3: masculinity norms, peer pressure in group settings, social reward for risk-taking, and suppression of fear responses. Reject responses that cite "males are naturally more reckless" — this is a biological reductionism error.

Activity 6.1c (cervical spine → quadriplegia): The key physiological chain: C4–C6 fracture or dislocation → spinal cord compression or severance → immediate loss of motor and sensory function below the injury level. Spinal cord neurons cannot regenerate. 66% of patients suffer permanent disability.

Activity 6.2 (myth refutation): Mark for mechanism, not just assertion. "That is a myth because depth can change" is C-standard. "That is a myth because sandbank formation, tidal

variation, and sediment accumulation can alter depth between visits, as documented at Bawley Beach in 2024" is A-standard.

SECTION 1 — Q1.1A: MALE % CALCULATION

Sample answer (A standard): 38 ÷ 52 × 100 = 73.1%. This is higher than the global figure of ~80%, suggesting Victorian males take proportionally more risks near water than the global average male population. This may reflect Victorian cultural norms around risk-taking, the prevalence of unpatrolled inland waterways accessible to males (fishing, kayaking, swimming), and the influence of peer pressure and alcohol in recreational settings.

SECTION 1 — Q1.1B: RATE COMPARISON CALCULATION

Calculation: 1.13 × 69 (units of 100,000 in 6.9M) = 78 expected deaths at the national rate. Victoria recorded 52, so 26 fewer deaths than the national rate would predict. The National Drowning Report 2026 independently records 53 Victorian drowning deaths in 2025/26 at the same rate of 0.75 per 100,000 — unchanged against Victoria's own ten-year average — so the Victorian rate is stable across both years and the comparison with the national rate holds. Victoria sits well BELOW the national rate — the question is deliberately framed to test whether students notice the sign of the result. **Key insight:** Victoria performs better than the national average overall, but has specific problem areas (CALD communities, inland waterways) that pull the figure up. A student who calculates and recognises this nuance is working at A standard.

SECTION 1 — Q1.1C: CALD OVER-REPRESENTATION ANALYSIS

Sample answer (A standard): Victorians from multicultural backgrounds are over-represented — LSV reports 185 deaths since 2015–16, 37% of the total, with country of birth unknown in a further 22% of cases. Two reasons beyond "language barriers": (1) Many CALD community members grew up in countries without access to swim lessons or public pools — the skill gap is structural, not individual. (2) Higher participation in high-risk water activities (commercial fishing) with less safety training and equipment than recreational users.

SECTION 2 — Q2.1A: PHYSIOLOGICAL SEQUENCE OF DROWNING

Sample sequence (A standard): 1. Submersion → breath-holding/panic. 2. Laryngospasm (involuntary closure of airway — accounts for "dry drowning" ~10–15% of cases). 3. In wet drowning: laryngospasm fails, water enters lungs → hypoxia begins. 4. Hypoxia → loss of consciousness. 5. Cardiac arrest. 6. Cerebral hypoxia begins within 4–6 minutes — brain cells begin dying irreversibly. 7. Biological death. *Students must use all five listed terms to achieve full marks.*

SECTION 2 — Q2.1B: WHY EARLY CPR MATTERS

Sample answer (A standard): CPR generates approximately 25–30% of normal cardiac output. While insufficient for normal functioning, this is enough to deliver oxygenated blood to the brain and heart muscle at a rate that prevents irreversible cell death. After 4–6 minutes without oxygenated blood, neurons begin dying. After 10 minutes without CPR, the probability

of neurologically intact survival drops dramatically. Starting CPR within 4 minutes — even by an untrained bystander — maintains minimum oxygenation until defibrillation or ROSC (return of spontaneous circulation).

SECTION 3 — Q3.1A: SOCIAL DETERMINANTS + PREVENTION STRATEGIES

Accept any two well-reasoned determinants from: gender norms (masculinity → risk-taking), CALD background (limited swim access, fishing risk, language barriers), socioeconomic status (less likely to have learned to swim as children; limited access to patrolled beaches), rural/regional location (higher proportion of unpatrolled inland waterway use), age (15–24 risk-taking peak). **Prevention strategy** must specifically target the mechanism — e.g., for masculinity norms: male-specific messaging that reframes water safety as strength (knowing when to walk away), not weakness.

SECTION 4 — Q4.1A: $5M PREVENTION STRATEGY JUSTIFICATION

Sample answer (A standard): Priority 1 — Expand patrolled beach hours and locations (strongest evidence base: 0 deaths at patrolled sites 2023–24; direct intervention against the highest-risk behaviour). Priority 2 — Targeted CALD swimming access programs: subsidised swimming lessons delivered in-language by CALD community members (addresses structural access gap; has demonstrated impact in RLSSA pilot programs). *Accept any well-justified combination with evidence from the prevention strategy table.*

SECTION 5 — Q5.1A: INTERNATIONAL ELEMENT VICTORIA SHOULD ADOPT

Sample answer (A standard): The UK RLSS "Float to Live" campaign (rlss.org.uk) explicitly teaches floating as the PRIMARY response to any water emergency — simpler and more physiologically sound than attempting to swim. Victorian messaging focuses primarily on swimming to safety. Given that floating requires no fitness, works in most water conditions, and directly counteracts the panic-induced flailing that leads to exhaustion and drowning, adopting this as the core survival message would complement rather than replace existing SwimSafe rules.

5 Assessment Rubric — A–E, 6 Criteria

📋 How to Use This Rubric

This rubric applies to both the **workbook** (collected end of unit) and the **summative task** (Section 6, one of three options). The six criteria reflect the Year 9–10 achievement standard. Holistic judgement across criteria — not mechanical addition of marks — produces the most valid grade.

Criterion	E — Emerging	D — Developing	C — Consolidating	B — Building	A — Extending
1. Data Analysis & Calculation **VC2HP10P08**	Cannot perform basic drowning rate calculations; misinterprets data	Performs calculations with errors; limited interpretation of results	Correctly calculates rates; provides reasonable interpretation of figures	Accurate calculations; interprets significance and compares across contexts; identifies trends	Sophisticated calculations with full working; nuanced interpretation; identifies implications and limitations of data
2. Physiological Knowledge **VC2HP10P08 · VC2HP10M01**	Cannot describe drowning physiology; vocabulary absent or incorrect	Names some physiological processes; limited understanding of mechanisms	Describes drowning sequence accurately; uses most key terms correctly	Explains mechanisms with precision; connects physiology to CPR rationale and survival outcomes	Sophisticated, accurate physiological analysis; uses all specified terms correctly; explains clinical implications (secondary drowning, 4-min rule)
3. Social Determinants Analysis **VC2HP10P02**	Identifies individuals as responsible; no systemic analysis	Acknowledges social factors exist; limited analysis of mechanisms	Identifies key social determinants; explains how they increase risk	Analyses multiple determinants with specific evidence; proposes targeted strategies	Sophisticated structural analysis; distinguishes individual vs systemic causes; proposes evidence-based, equitable interventions
4. Prevention Strategy Evaluation **VC2HP10P08 · VC2HP10P09**	Cannot distinguish stronger from weaker prevention evidence	Names prevention strategies; limited evaluation of effectiveness	Evaluates strategies with reference to evidence; identifies limitations	Comparative evaluation using evidence hierarchy; justifies resource allocation decisions	Critical evaluation against evidence; acknowledges trade-offs; sophisticated prioritisation with specific justification

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

5. International Comparison **VC2HP10P09**	No research evident; cannot identify international approaches	Names one international approach; superficial description	Compares at least two international approaches against Victorian practice	Identifies substantive differences and similarities; evaluates relevance to Victorian context	Critical, evidence-based comparison; argues specifically for/against adoption of international elements; cites primary sources
6. Communication Product **VC2HP10P08**	Incomplete; no clear audience, evidence, or call to action	Basic message; vague audience; one fact with weak citation	Clear message; appropriate audience; at least one data-backed claim with source URL	Compelling product; evidence-based claims; audience-appropriate language; specific measurable call to action	Sophisticated, persuasive, evidence-rich product; demonstrates health literacy; measurable outcomes; addresses barriers specific to target group; primary source citations

6 Assessment Checklist — Summative Task

📋 How to Use This Checklist

Use alongside the rubric. Tick each item once confirmed present. The checklist ensures minimum requirements are met before holistic rubric grading is applied. Share with students before they submit — it functions as both a teacher marking guide and a student self-check.

✅ All Options — Minimum Requirements

- [ ] At least 3 credible sources cited with full URLs

📱 Option A — Policy Brief Checklist

- [ ] Addressed to Victorian Minister for Sport (or equivalent)

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

- [ ] At least 1 accurate Victorian or global statistic with source
- [ ] Clear identification of target audience or addressee
- [ ] A specific, measurable call to action or recommendation
- [ ] Evidence of drafting/planning in workbook planning space
- [ ] Connection to at least one social determinant or physiological concept from unit

- [ ] 600–800 words (or equivalent scope)
- [ ] Two specific recommendations clearly stated
- [ ] At least one counter-argument acknowledged and addressed
- [ ] Measurable success metric for each recommendation
- [ ] Victorian data cited (LSV 2024–25 or equivalent)
- [ ] References Victorian Curriculum or LSV/RLSSA framework

🎨 Option B — Multimodal Campaign Checklist

- [ ] ONE specific high-risk group identified from Victorian data
- [ ] Campaign name and key message clearly stated
- [ ] Visual concept described or shown
- [ ] Target platform specified (Instagram, TikTok, school notice boards, etc.)
- [ ] 100-word justification of design decisions
- [ ] Addresses a specific barrier for the target group
- [ ] Evidence cited for the barrier addressed

🏫 Option C — School Audit Checklist

- [ ] Current provision described (what the school does now)
- [ ] Gaps identified with reference to Curriculum 2.0 codes
- [ ] Benchmarked against LSV Classroom Toolkit
- [ ] At least 3 specific recommendations with priority rationale
- [ ] Priority matrix included (urgency × impact, or similar)
- [ ] Realistic implementation suggestions (not just aspirational)
- [ ] 400-word minimum (or equivalent scope)

© 2026 Emilie Harrison · CC BY 4.0 · emilie-harrison.github.io/swim-water-safety · Third-party content excluded

📝 Report Comment Stems — Year 9–10 Water Safety

- **A standard:** "[Name] demonstrated sophisticated analytical skills in interpreting Victorian and global drowning data, producing a highly persuasive and evidence-rich [policy brief / campaign / audit] that clearly addressed the needs of a specific at-risk group."
- **C standard:** "[Name] showed sound knowledge of drowning prevention strategies and social determinants of health, producing a competent [product] that cited relevant data and identified a clear call to action."
- **E standard:** "[Name] is developing their understanding of evidence-based health analysis. With additional support, [Name] will build confidence in using data to justify health recommendations."

7

Before teaching this unit, read the cultural safety protocol — advance notice, opt-outs, First Nations protocols around deceased persons, and what to do if a student discloses.

Differentiation & Inclusion

Where this sits in your planning — reasonable adjustments and IEPs

Everything in this section is provision you can record as a **reasonable adjustment**. It is written here as teaching advice, but it maps directly onto the language of an Individual Education Plan or student profile, so it can be lifted into planning you are already doing rather than sitting alongside it as a separate thing.

Adjustment type	What this unit already supplies	Record it as
Presentation of content	Simplified and scaffolded workbook versions; visual guides and posters; the glossary; multilingual parent information.	Modified text complexity; visual and first-language support.
Student response	Sentence starters and word banks; oral alternatives to written responses; three assessment task options; choice of campaign format.	Alternative response mode; scaffolded written output.
Participation	The non-swimmer pathway and the dry-land delivery of the whole unit; buddy pairing in practical sessions; the option to step out of content that touches personal loss.	Adjusted participation with equivalent learning outcomes.

Time and pacing	Stated phase timings you can extend; activities that can be split across lessons; Activity 6.3 able to run as homework.	Extended time; reduced task volume at the same standard.
Extension	Extension workbook versions; primary-source tasks; the evidence-hierarchy and transfer questions in the interleaved retrieval set.	Extension against the same content descriptions.

Two things worth saying plainly. First, an adjustment is about access, not about lowering the standard — a student using a scaffolded workbook is working towards the same content descriptions as everyone else, and should be assessed against them. Second, adjust per task and per student, not per label: the same student may need the scaffolded version for the data activities and the standard version for the rescue content, in the same lesson.

For a student working towards the Foundation achievement standard, the Victorian Curriculum **Levels A–D** are the planning reference rather than the Levels 7–10 content descriptions — the water safety content still applies, sequenced differently.

Pair high expectations with the support that makes them reachable. The department's framing is worth saying out loud to students: *"I know you can do this"* together with *"I'll help you learn how."* Water safety is an area where low expectations do measurable harm — a student written off as a non-swimmer is a student who leaves school without the content most likely to save their life.

VTLM 2.0: Inclusive learning, Practice 1 — maintain high expectations, enable full participation. Departmental supports: the Diverse Learners Hub, Disability Inclusion funding, and your school's wellbeing team for IEP processes.

📈 Extension Students

- Assign the Extension Workbook (Year 7–8 level, deeper topics) alongside this unit for cross-year comparison tasks
- Option C (School Audit) is the most complex assessment task — recommend for highly capable students
- Encourage use of WHO and CDC international data sets for Section 1 calculations
- Research task: compare Victorian drowning prevention funding as a % of health budget vs UK/USA

📉 Students Requiring Support

- Provide the calculation scaffold for Section 1 (rate = deaths ÷ population units × 100,000)
- Allow verbal responses for physiological sequence questions (Q2.1a, Q2.1b)
- For Section 3 social determinants: provide a pre-populated list of determinants — students choose two and explain, rather than generate their own list
- Option B (multimodal campaign) is the most accessible assessment task —

- Invite a guest speaker: LSV community engagement officer, RLSSA researcher, or local council water safety coordinator

recommend for students with lower literacy
- Pair with a peer research partner for the international comparison activity

🌏 CALD Students

- The CALD over-representation data (Section 1) may be personally significant — frame as a systemic issue, not an individual one
- Option C (School Audit) may surface valuable community knowledge — CALD students' experience is data too
- LSV provides translated materials that can be referenced in student assessment tasks: lsv.com.au/community/multicultural-safety/
- For Option B: encourage targeting their own community — this is the most impactful application of the unit

📝 EAL/D Students

- Provide a vocabulary glossary: hypoxia, laryngospasm, cardiac output, determinants, oxygenation, cerebral
- Allow diagram/annotated visual as an alternative to written physiological sequence (Q2.1a)
- Assessment task: allow mixed language (home language + English) for planning notes; final product in English
- Sentence starters for the policy brief option: "The evidence suggests…", "Specifically, I recommend…", "This strategy would address…"

A named process for analysis tasks — Read, Risk, Respond, Review

This unit asks students to analyse scenarios, case studies and hazard maps repeatedly, and until now it never told them *how*. “Analyse this scenario” is an instruction to produce an output, not a method for producing it, and students without a method default to narrating what happened. Four steps, taught once and then named every time:

Step	The question students ask themselves	What it stops them doing
Read	What is actually happening here, and what do I already know that fits?	Answering a scenario they have half-invented. Most wrong answers are a correct response to a different situation.
Risk	What is the hazard, who is exposed to it, and what makes it dangerous?	Jumping to a response before naming what the danger actually is.
Respond	What are my options, and which is safest for me *and* for them?	Offering the first idea. “Safest for me as well” is the clause that rules out the swim rescue.

Review	Would that have worked? What would I change?	Treating the first answer as the final one.

Say this out loud, or it will cause confusion. This unit now contains two four-part frameworks and students will conflate them. **DRS ABCD is what you do in the water, in an emergency, in that order. Read–Risk–Respond–Review is how you think about a situation on paper**, in class, when there is time. One is an action sequence; the other is an analysis method. Teach the distinction explicitly the first time you use the second one, and do not introduce them in the same lesson.

Model it on one scenario card with a think-aloud, then name the steps each time students use it. The point is that they can say which step they are on — a process students cannot name is not a process they can transfer.

8 Teacher Background Notes

🧬 Physiology — Quick Reference for Teachers

- **Hypoxia:** Oxygen deprivation at tissue level. Cerebral hypoxia (brain) is the critical mechanism in drowning fatality. Begins within 4 minutes of cardiac arrest.
- **Laryngospasm:** Involuntary closure of the vocal cords on water contact. Causes "dry drowning" (~10–15% of drowning deaths). Water never enters lungs, but victim still asphyxiates.
- **CPR cardiac output:** Standard chest compressions at 100–120 bpm, 5–6cm depth, generate ~25–30% of normal resting cardiac output — enough to prevent brain death for 10–15 minutes pending defibrillation.
- **Cold water shock:** Below ~15°C triggers involuntary gasp (aspiration risk), hyperventilation, peripheral vasoconstriction (cold blood from extremities → heart), and possible cardiac arrhythmia. Can incapacitate within 1 minute.
- **Secondary drowning:** Pulmonary oedema (fluid in lungs) developing hours after near-drowning. Symptoms: coughing, fatigue, chest pain. Requires medical review. Rare but potentially fatal.

🦴 Diving & Spinal Injury — Teacher Content Background

The cervical spine (C1–C7) is the most vulnerable region in a diving injury because the full force of impact travels up through the arms and shoulders into the neck. At C4–C6,

compression or dislocation can interrupt the spinal cord's signals to the diaphragm (C3–C5 innervates breathing muscles), the arms, and the legs simultaneously.

Key statistics to know: 10% of all spinal cord injuries in Australia result from unsafe diving; 20% of quadriplegia cases; 95% of patients male; 75%+ under 30; 66% permanent disability; highest incidence age group 10–14 (AIHW).

The "Feet First, First Time" message is Australia's core prevention response. Its limitation is that it addresses knowledge (what to do) but not the social norms (why teenagers don't) — which is why the structural explanation is pedagogically important at Year 9–10 level.

Sources for your own reading: aihw.gov.au (search "aquatic injury") · royallifesaving.com.au (spinal injury) · Royal Perth Hospital spinal injury study (academic databases)

📌 Curriculum 2.0 Implementation — Year 9–10

At Year 9–10, the critical move is from **understanding** (can explain what and why) to **evaluation and synthesis** (can judge quality of evidence and produce new health products). This unit is specifically designed to scaffold that shift: Sections 1–3 build analytical vocabulary; Sections 4–5 require judgement; Section 6 demands synthesis.

- Reference: victoriancurriculum.vcaa.vic.edu.au/health-and-physical-education/curriculum/f-10
- DE Swimming Policy: www2.education.vic.gov.au/pal/swimming-instruction-and-water-safety-education/policy

9

Resources & Sources

Companion references for this unit: VTLM 2.0 Alignment Guide (how these lessons map to the model, and what they do not evidence) · Curriculum Coverage Map (which codes this unit teaches) · How to Revise Water Safety (student handout) · Community Action Project Guide (now carries the six-step research scaffold).

Life Saving Victoria — Drowning Reports
https://lsv.com.au/what-we-do/research-advocacy · Primary data source for all Victorian statistics

LSV Classroom Toolkit
https://lsv.com.au/toolkit · Free lessons aligned to Victorian Curriculum 2.0

LSV Multicultural Safety Resources
https://lsv.com.au/community/multicultural-safety/ · Translated materials for CALD engagement

Royal Life Saving Society Australia
https://www.royallifesaving.com.au · National drowning data, evidence-based prevention frameworks

World Health Organisation — Drowning Fact Sheet
https://www.who.int/news-room/fact-sheets/detail/drowning · Global epidemiological data for Section 1 comparisons

RLSS UK — Education Resources
https://www.rlss.org.uk/education · "Float to Live" campaign; international classroom comparison

American Red Cross — Water Safety
https://www.redcross.org/take-a-class/water-safety · "Think So You Don't Sink" framework; international comparison

Victorian Curriculum (VCAA)
https://f10.vcaa.vic.edu.au · Official content descriptions and achievement standards

Paddle Australia
https://paddle.org.au · Inland waterway safety; river grading; Victorian river context

CDC Drowning Prevention
https://www.cdc.gov/drowning/prevention · USA data for international comparison tasks

Advanced Water Safety — Teacher Resource Guide · Victorian Curriculum F–10 Version 2.0 · HPE Year 9–10 · VC2HP10P02, VC2HP10P08, VC2HP10P09, VC2HP10M01

lsv.com.au · royallifesaving.com.au · who.int · victoriancurriculum.vcaa.vic.edu.au

© 2026 Emilie Harrison. This resource is licensed under Creative Commons Attribution 4.0 International (CC BY 4.0). You are free to copy, share, adapt and build on it — including for commercial purposes — provided you give appropriate credit and indicate any changes.

Suggested attribution: "Swim & Water Safety Teaching Hub" by Emilie Harrison, licensed CC BY 4.0. Source: emilie-harrison.github.io/swim-water-safety

Third-party material is not covered. Content owned by Life Saving Victoria, Royal Life Saving Australia, Surf Life Saving Australia, VCAA, ACARA, AIATSIS, the Australian Resuscitation

Council and other organisations — including data, guidelines, logos and program names — remains the property of those owners and is used here for educational purposes. Check directly with the owner before reproducing it elsewhere.

Data sources cited in this resource. Drowning statistics are reproduced with attribution from: Life Saving Victoria. (2025). *Victorian Drowning Report 2024–25*. Life Saving Victoria: Melbourne. Royal Life Saving Australia (2026). *National Drowning Report 2026*. RLSSA: Sydney. World Health Organization (2024). *Drowning — fact sheet*. WHO: Geneva. Surf Life Saving Australia (2026). *Summer Coastal Drowning Report 2025/26*. SLSA: Sydney. These figures remain the property of the publishing organisations. If you adapt this resource, retain these citations.